

HOW TO OPEN AN ASSISTED LIVING HOME FOR ADULTS WITH BRAIN INJURY in Texas

An HHSC-licensed Type B Assisted Living Facility — funded through STAR+PLUS.

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact & fee should be re-verified before you rely on it.

The Texas model — read this first

In Texas, a home that cares for adults with brain injury is licensed by the Texas Health and Human Services Commission (HHSC), Long-Term Care Regulation, as an Assisted Living Facility (ALF) under Texas Health & Safety Code Chapter 247 and the rules at 26 Texas Administrative Code Chapter 553. Because brain-injury residents usually cannot evacuate a building by themselves and may need help overnight, you will almost always license as a Type B ALF (the type built for residents who are not capable of self-preservation). You get paid mainly through STAR+PLUS — Texas Medicaid's managed-care long-term-care program — by enrolling as a Texas Medicaid provider and then contracting with the STAR+PLUS health plans. This guide walks that whole path, broken down so a first-timer can follow it.

Accreditation — strongly recommended, and how we build. Texas does not require national accreditation to hold an assisted living license, but for a brain-injury program it is a real advantage: CARF (which offers a Brain Injury specialty) and The Joint Commission signal quality to STAR+PLUS health plans, hospital discharge planners, and families, can strengthen your contracts, and are sometimes required by post-acute brain-injury programs. Every PolicyReady product is built to those standards behind the scenes. Treat accreditation as a goal from day one (see Parts 7 & 9).

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It is written for someone who has never opened a facility — where we name a thing (an agency, a form, a rule, an insurance type), we break it down and tell you exactly where to go and what to ask. It is general information, not legal, tax, or clinical advice; verify each step with HHSC, the Texas Medicaid & Healthcare Partnership (TMHP), and the STAR+PLUS health plans (see Part 11) before you rely on it.

The journey at a glance

Ten milestones take you from idea to a licensed, contracted, paid home. Each part below is one rung.

1	Decide this is your business	A licensed, 24-hour Type B ALF — durable, census-based revenue (Part 1)
2	Form the business & insure	Entity, EIN, bank, and the right insurance — broken all the way down (Part 3)
3	Secure & prepare the home	Zoning, and the 26 TAC 553 physical-plant specs (Parts 3–4)
4	Train your manager	The HHSC-approved 24-hour ALF manager course (Part 5)
5	Build & train staff	Orientation, on-the-job training, first aid/CPR, background checks (Part 5)
6	Write your policies	P&P Manual + the operating standards of 26 TAC 553 (Parts 4–6)
7	Get the HHSC license	Apply in TULIP; pass the Life Safety

		Code + health surveys (Part 6)
8	Enroll & contract for STAR+PLUS	TMHP/PEMS enrollment + contract the STAR+PLUS health plans (Part 8)
9	Get your first residents	MCO service coordinators + hospital/TIRR discharge planners (Part 10)
10	Who to call & what to ask	Every contact + the exact questions, all in one place (Part 11)

Rule of thumb. Start your manager's 24-hour course and your fire-marshal conversation early — the Life Safety Code survey is where new ALFs get held up. Do the cheap, reversible steps (form the entity, sign a contingent lease) first, and get everyone's background checks started right away.

Before you begin — a realistic picture

Be honest with yourself about what this is. You are not opening a boarding house; you are becoming a licensed 24-hour care provider for adults whose judgment, memory, mobility, or safety awareness has been changed by a brain injury. That means real regulatory scrutiny (a fire-side Life Safety Code survey and a health survey before you ever get paid), real staffing (an attendant in the building whenever residents are present, plus awake overnight coverage for Type B), and real money tied up before the first Medicaid dollar arrives — the home and its life-safety upgrades, the manager's course, insurance, and background checks all come before revenue. Most first-timers underestimate two things: how long licensing plus MCO contracting takes end-to-end (plan for several months, not weeks), and how much the fire sprinkler / Life Safety Code question can make or break a specific house. The upside is that the path is defined and the Type B license is built precisely for these residents — if you work the steps in order and verify each fee and contact as you go, it is genuinely doable.

Roadmap of the Parts: Part 1 defines the business and why brain injury means a Type B license; Part 2 introduces every regulator and payer you'll deal with. Parts 3 and 4 are the hands-on build — forming the entity, insurance, securing and zoning the home, and meeting the 26 TAC 553 physical-plant and Life Safety Code specs. Part 5 covers your manager, staff, and required training; Part 6 walks the HHSC license application in TULIP. Part 7 follows a resident from referral through discharge; Part 8 is how you actually get paid through STAR+PLUS. Part 9 lays out costs and timing (and accreditation), Part 10 is how you land your first residents, and Part 11 collects every contact and every “verify” in one place. Read it once through, then work it as a checklist.

PART 1

What This Business Is

In short: An HHSC-licensed Assisted Living Facility — almost always a Type B — for brain-injury adults who can't evacuate on their own and may need overnight care. It's a real-estate + 24-hour-care business.

Texas licenses assisted living facilities and sorts them by how much help residents need in an emergency. That single distinction — can the resident get out of the building alone? — is what puts a brain-injury home in the Type B category.

The license types (26 TAC § 553.5)

Type	Who it's for
Type A	Residents who can evacuate WITHOUT physical staff help, don't need routine attention overnight, and can follow directions in an emergency
Type B	Residents who NEED staff help to evacuate, may need attention during nighttime sleeping hours, and may not be able to follow emergency directions (but are not permanently bedfast) — the brain-injury fit
Type C	A separate 4-bed adult foster care category; new Type C licenses are no longer issued

Why brain injury = Type B

The A/B line is about “capacity for self-preservation” — whether a resident can get themselves out and respond correctly in an emergency without a staff member physically assisting. Many brain-injury adults cannot: they may have impaired mobility, slowed processing, poor safety awareness, or need overnight supervision. That places them, by definition, in Type B. Licensing Type B means your building and staffing are held to the higher life-safety standard (see Parts 4–5), which is exactly what these residents need.

There is no separate “memory care” or “brain injury” license in Texas. You serve brain-injury residents under a standard Type B ALF license. If you also lock exits or serve residents with Alzheimer's/dementia, you can add a voluntary Alzheimer's certification (Form 3721) with extra staff training — but the base license is the Type B ALF.

“Not permanently bedfast.” A Type B ALF may not keep a resident who is permanently bedfast — that person needs a nursing facility. A brain-injury resident who can be transferred and is not permanently confined to bed fits Type B. Build your admission criteria around this line (Part 7).

Key terms. HHSC = Texas Health & Human Services Commission (your licensor). LTCR = Long-Term Care Regulation (the HHSC unit that licenses/surveys you). ALF = Assisted Living Facility. TULIP = the online licensing portal. STAR+PLUS = Texas Medicaid managed long-term care. HCBS = Home & Community-Based Services (the waiver that pays for assisted living). MCO = Managed Care Organization (a STAR+PLUS health plan). TMHP = Texas Medicaid & Healthcare Partnership. PEMS = the Medicaid provider-enrollment system. NAR/EMR = Nurse Aide Registry / Employee Misconduct Registry. CRS = Comprehensive Rehabilitation Services (the state TBI program). NPI = National Provider Identifier. CARF/TJC = the two accreditors.

PART 2

Who Regulates You (and What Each One Does)

In short: HHSC licenses you, Texas Medicaid (TMHP) enrolls you and the STAR+PLUS health plans pay you, the fire marshal clears your building, and the Long-Term Care Ombudsman advocates for residents.

HHSC — Long-Term Care Regulation (your licensor)

The Health and Human Services Commission, through its Long-Term Care Regulation division, issues your assisted living license, writes the 26 TAC Chapter 553 standards, runs the TULIP online application system, and conducts the pre-licensure Life Safety Code and health surveys plus ongoing inspections and complaint investigations. Policy, Rules & Training: (512) 438-3161.

Texas Medicaid / TMHP and the STAR+PLUS health plans (who pay you)

The Texas Medicaid & Healthcare Partnership (TMHP) is where you enroll as a Medicaid provider (through the PEMS system). STAR+PLUS is the managed-care program that actually pays for assisted living — you then contract with the STAR+PLUS Managed Care Organizations (MCOs) serving your area, and each member's MCO service coordinator authorizes and arranges the service. TMHP: 1-800-925-9126.

The local fire marshal & building/health authorities

Before HHSC licenses you, your local fire marshal inspects the home against the adopted NFPA 101 Life Safety Code for your facility type, and your city/county handle zoning and building permits. For a brain-injury (Type B) home whose residents can't self-evacuate, this inspection matters more, not less — bring the fire marshal in early (Part 4).

Long-Term Care Ombudsman & Adult Protective Services

The Texas State Long-Term Care Ombudsman is a free, independent advocate for residents of assisted living and nursing facilities — you post their number for residents (1-800-252-2412; ltco.texas.gov). Adult Protective Services (through DFPS) takes abuse/neglect/exploitation reports, and HHSC maintains the Nurse Aide Registry and Employee Misconduct Registry you must check on staff (Part 5).

PART 3

Form Your Business & Secure the Home — Step by Step

In short: *This is where people get stuck. We break each piece down: what it is, where to go, and exactly what to do.*

3.1 Form your legal entity

An “entity” is the legal business (usually an LLC) that holds the license and bank account and shields your personal assets. In Texas you form it at the Texas Secretary of State.

1. **Go to the Texas Secretary of State.** File online through SOSDirect/SOSUpload at sos.state.tx.us, or by mail/in person. Main business line: (512) 463-5555.
2. **File a Certificate of Formation — LLC (Form 205).** The filing fee is \$300 (credit-card payments add a 2.7% convenience fee). Name the LLC and list a registered agent (receives legal mail — can be you).
3. **Adopt an Operating Agreement** — the internal document setting out ownership, management, and money. Texas doesn't file it, but your bank and partners will want it.
4. **Save your documents and your filing number** — the bank, TMHP, and HHSC will all ask for them.

Texas has no LLC newspaper-publication step. Unlike some states, Texas does not make you publish a formation notice. But an LLC must file a yearly Franchise Tax report and Public Information Report with the Texas Comptroller (most small entities owe no tax but still must file). If you operate under a different name, file an Assumed Name Certificate (Form 503).

3.2 Get your EIN (federal tax ID)

An EIN is your business's federal tax number — free from the IRS, and you need it for the bank, payroll, and Medicaid enrollment.

1. **Go to IRS.gov** and search “Apply for an EIN online” (the EIN Assistant). It's free — never pay a third-party site.
2. **Apply online** with the responsible party's name and SSN/ITIN and your entity info.
3. **Save the confirmation (CP-575)** — you get the EIN immediately.

3.3 Open a business bank account

What to have ready:

- EIN confirmation from the IRS
- Your Certificate of Formation (Form 205) and filing number
- Operating Agreement (LLC)
- A Beneficial Ownership form (owners of 25%+ and a control person)
- Government photo ID, SSN, DOB, home address for each owner/signer
- An opening deposit (often ~\$100)

Where to go: a major bank with lots of Texas branches — Chase, Bank of America, Wells Fargo, Frost Bank (a large Texas-based bank), or a local credit union. Compare monthly fees and how to waive them, transaction/cash-deposit limits, branch access, online bill pay, and whether they offer a business line of credit.

3.4 Get your insurance — what you need, who sells it, and what to ask

An assisted living home cares for vulnerable adults in a building, so you need several kinds of coverage — not one policy:

Coverage	What it protects against	Must-have?
General Liability (GL)	A visitor/family slip-and-fall or property damage on your premises	Yes — foundational
Professional Liability (malpractice / E&O)	Claims about the CARE — supervision, med errors, neglect	Yes — top healthcare coverage
Abuse & Molestation	Allegations of abuse by staff or others (GL)	Yes — high exposure

	excludes this!)	
Commercial Property	The building, contents, equipment; add Business Interruption	Yes if you own; often if you lease
Commercial Auto (incl. hired/non-owned)	Transporting residents; staff driving own cars for work	Yes if you drive residents
Workers' Compensation	Staff injuries (lifting/transferring is high-risk)	Optional in TX, but strongly advised — verify
Cyber Liability	A breach of protected health information (HIPAA)	Strongly recommended

A Texas quirk on workers' comp. Texas is the one state that does not require most private employers to carry workers' compensation. You can be a “non-subscriber,” but that exposes you to injury lawsuits with fewer protections — for a home where staff lift and transfer residents, most operators carry it anyway. Discuss the trade-off with your agent and attorney.

Real places to get it

Buy through an independent agent/broker who accesses specialty carriers. Carriers that write senior-care/residential-care risk include Philadelphia Insurance (PHLY), CNA, Markel, Berkshire Hathaway GUARD, Crum & Forster, Selective, and Nationwide. Specialty programs/brokers for this niche include Caitlin Morgan, Gallagher, AssuredPartners, Professional Insurance Group, and XInsurance. To find a local Texas agent, use the “Trusted Choice” Find-an-Agent directory at [trustedchoice.com](https://www.trustedchoice.com) (filter healthcare/senior-care), or ask a licensed peer. Verify each carrier writes assisted living in Texas before relying on a quote.

What to ask the insurance agent

“Do you write Type B assisted living facilities / brain-injury homes — which carriers?”

“Is professional liability included — claims-made or occurrence — and what limits?”

“Is abuse & molestation included, at what limit, sub-limit or full?”

“Are GL and professional liability shared limits or separate?”

“What limits does a STAR+PLUS MCO or my landlord/lender require?”

“Should I subscribe to workers' comp, or is a non-subscriber plan smarter for me?”

3.5 Secure & zone the home

- ☐ Call your city/county PLANNING & ZONING office: is a residential assisted living use for [n] residents allowed at this address, or do you need a permit? Texas group homes for people with disabilities generally have residential-use protection under fair-housing law — ask them to confirm in writing.
- ☐ Confirm the building can meet the 26 TAC 553 physical-plant specs (Part 4) and be made accessible.
- ☐ Confirm the home can be brought up to the NFPA 101 Life Safety Code for a Type B ALF — including fire sprinklers if you're a new small facility (Part 4).
- ☐ If leasing, make sure the lease allows your use and resident count, and make it contingent on licensure.

3.6 Map ownership + background checks

Identify every owner and controlling person — you disclose them on the HHSC application, and every employee must clear a criminal-history check and the Nurse Aide Registry / Employee Misconduct Registry before contact with residents (Part 5.4).

3.7 Get your NPI (National Provider Identifier)

An NPI is a free national ID payers use to identify you. Your FACILITY needs a Type 2 (Organizational) NPI:

1. **Create an I&A account** at nppes.cms.hhs.gov (Identity & Access) for the person who'll manage the number (usually an owner).
2. **Log into NPPES** and start a new application; choose Type 2 (Organizational).

3. Enter your legal name, EIN, and address and pick the taxonomy that fits an assisted living / residential facility — confirm the exact taxonomy TMHP wants.

4. Submit — it's free and usually processes in about 10 business days. Save your NPI; you'll need it for Medicaid enrollment (Part 8).

***One NPI trap that delays Medicaid enrollment.** The name and address on your Type 2 NPI must match your entity exactly as it appears on your HHSC license and your IRS EIN letter — a comma, a “LLC” vs “L.L.C.”, or an old address will bounce your PEMS enrollment and cost you weeks. Set the NPPES record straight before you file with TMHP.*

3.8 Put the paper trail in one place

Every agency in this guide — the bank, TMHP, the STAR+PLUS plans, and HHSC surveyors — asks for the same core documents, so assemble them once in a labeled binder (paper and a backed-up digital folder) and you'll stop re-hunting for them.

Founding-documents checklist:

- ☐ Certificate of Formation (Form 205) + the SOS filing number and file-stamped copy
- ☐ Operating Agreement, and a current ownership/controlling-person list with percentages
- ☐ IRS EIN confirmation letter (CP-575)
- ☐ Type 2 (Organizational) NPI confirmation
- ☐ Assumed Name Certificate (Form 503), if you operate under a DBA
- ☐ The insurance binder/declarations pages for every policy in 3.4
- ☐ The lease (with your use and resident count) or the deed, plus zoning confirmation in writing
- ☐ Photo ID, SSN, and home address on file for each owner/signer

PART 4

The Home — Physical Plant, Safety & Life-Safety Code (26 TAC 553)

In short: Texas gives you real numbers for space and bathrooms, and holds your building to the NFPA 101 Life Safety Code. Here's what each requirement means and how the surveyor checks it — build and set up to these before you apply.

“Physical plant” just means the building itself and everything fixed in it — the rooms, plumbing, kitchen, and fire systems. HHSC inspects all of it (a Life Safety Code survey by the fire side, and a health survey) before it issues your license. The rules live in 26 TAC Chapter 553, and the fire rules adopt the 2012 NFPA 101 Life Safety Code. Below, each requirement is followed by what it means for you.

4.1 Bedrooms & space (26 TAC 553, Subchapter D)

- ☐ Single-resident bedroom: at least 100 sq ft of floor space
- ☐ Multiple-occupancy bedroom: at least 80 sq ft PER resident
- ☐ No more than 4 residents per bedroom, and no more than 50% of licensed capacity in bedrooms holding 3 or more residents
- ☐ Each resident has their own bed and personal storage; room portions under 10 ft in the smallest dimension don't count toward the minimum
- ☐ Windows: sill no higher than 44 inches above the floor, with glazing at least 8% of the required floor area
- ☐ A living/common area and a dining area sized for the residents

What this means for you: measure every bedroom before you sign a lease. A single-occupant room needs a full 100 sq ft (roughly 10 by 10), more than some states require — a small bedroom that would pass elsewhere may not pass here. If you plan shared rooms, remember the 50% cap: in a 10-bed home, no more than 5 beds can sit in rooms of three or more. Brain-injury residents often use wheelchairs or walkers, so leave real turning room beyond the minimum.

4.2 Bathrooms & water

- **Toilet rooms:** each bedroom must be served by a private, connecting, or general toilet room; in mixed-gender facilities, provide gender-specific or single-occupant toilet rooms. Confirm the exact fixture-per-resident ratio for your size/type in the applicable 553 section.
- **Safety:** grab bars at toilets and in tubs/showers, non-slip surfaces, and features for residents using wheelchairs/walkers/transfer benches.
- **Hot water:** dish and laundry sanitizing water must reach 180°F; set resident bathing water to a safe, anti-scald temperature (verify the exact resident-fixture maximum for your section).
- **Kitchen:** kitchen and food-storage rooms must stay at or below 85°F at peak load; store, prepare, and serve food safely with proper refrigeration.

What this means for you: think like a surveyor walking your kitchen and bathrooms. The most common early citations are unsafe water temperature, missing grab bars, and food/chemical storage. Put a thermometer in the fridge and on the hot-water tap, add grab bars, and lock up chemicals and medications before you apply.

4.3 Fire & the Life Safety Code (the big one for Type B)

- ☐ The home is surveyed under the 2012 edition of NFPA 101; a small Type B facility falls under NFPA 101 Chapter 32 (New Residential Board & Care)
- ☐ Fire sprinkler system — required for a new small ALF seeking licensure or relocation on or after August 31, 2021
- ☐ Smoke detectors/alarms placed per the adopted NFPA 101 chapter; test and document them
- ☐ Clearly marked exits, and a written emergency and evacuation plan that accounts for residents who cannot self-evacuate

- ☐ Fire drills and staff trained to move non-ambulatory residents; keep every fire, building, and health inspection report in one dated compliance binder

Get the fire marshal in early — this is where Type B homes get held up. Because your residents can't evacuate themselves, the Life Safety Code survey is the make-or-break step. Call your local fire marshal while you're still choosing or modifying the home — the sprinkler requirement alone can decide whether a given house is workable. HHSC offers an expedited Life Safety Code inspection (about 15 business days); contact info is in Part 11.

4.4 Brain-injury environmental design (why it matters here)

Beyond the code minimums, a brain-injury home should be set up for the residents' specific risks: clear, low-clutter walking paths to reduce falls; consistent, labeled locations for personal items (memory and orientation support); secured exits or door alarms if any resident wanders (if you lock exits, look at Alzheimer's certification and the Life Safety Code egress rules); reduced noise and visual overstimulation in common areas; and a quiet space a dysregulated resident can go to. This is both good care and part of keeping residents safe under your Type B obligations.

4.5 Walk your own building first — a pre-inspection checklist

Before you invite the fire marshal or the HHSC health surveyor, walk the home yourself with this list and fix what you find. These are the items that most often generate early citations for a new Type B ALF, and every one is cheaper to catch now than on survey day.

Life-safety & environment:

- ☐ Fire sprinkler system installed and its inspection tag current (required for new small ALFs since Aug 31, 2021)
- ☐ Smoke alarms tested and dated; a written, drilled evacuation plan that names who moves each non-ambulatory resident
- ☐ Every exit unobstructed, clearly marked, and openable without a key from the inside
- ☐ Fire extinguishers charged, tagged, and mounted; emergency lighting works
- ☐ Hot-water tap at resident fixtures measured with a thermometer and set anti-scald; dish/laundry sanitizing water reaches 180°F
- ☐ Grab bars secured at every toilet and tub/shower; non-slip surfaces in wet areas
- ☐ Medications and cleaning chemicals locked and stored separately from food
- ☐ Refrigerator/freezer thermometers in place and logged; kitchen holds at or below 85°F at peak
- ☐ Bedrooms measured against the 100 / 80 sq ft minimums; the 4-per-room and 50%-of-capacity caps respected
- ☐ One dated compliance binder holding every fire, building, and health inspection report

PART 5

Staff, the Manager & Required Training — by Position

In short: Each role has its own age, training, and screening rules. Here they are, one by one — and the manager's course plus your background-check system are the pieces to start early.

5.1 Your ALF manager (26 TAC § 553.253)

Every assisted living facility must have a qualified, trained manager (who may be you). Here's exactly what that takes:

- 1. Meet the education floor.** A small-facility manager needs a high-school diploma or equivalent; a large-facility manager needs an associate's degree in nursing or health-care management, or a bachelor's degree, or a high-school diploma plus at least one year of management or health-care-industry management experience.
- 2. Complete the HHSC-approved manager course.** A management course of at least 24 hours, taken from a provider on HHSC's approved-course list (TALA and others deliver it). Complete it by your first anniversary as manager.
- 3. Add the 8-hour standards training.** Within the first 3 months of employment as manager, complete 8 hours of training on the assisted living standards (26 TAC 553).
- 4. Know the required content.** The course covers the assisted living standards; resident characteristics including dementia; resident assessment; basic management; food and nutrition; federal law with emphasis on ADA accessibility; community resources; ethics; and financial management.
- 5. Work the hours.** The manager must work at least 40 hours a week and be on duty, with a designated back-up when absent.
- 6. Keep up your CE.** 12 hours of continuing education each year.

Start this early. You can't operate without a qualified manager, and the 24-hour course plus the 8-hour standards training take real time to schedule and finish. Line up an HHSC-approved course before you apply. Approved-course list and TALA: tala.org.

5.2 Direct-care staff (attendants)

- **Age/education:** at least 18 years old or a high-school graduate.
- **Orientation:** every staff member completes at least 4 hours of orientation before assuming any job responsibilities.
- **On-the-job training:** attendants complete 16 hours of on-the-job supervision and training within their first 16 hours of employment after orientation.
- **Annual training:** direct-care staff complete at least 6 documented hours of continuing education each year — which must include 1 hour on fall prevention and 1 hour on behavior management.
- **Coverage:** an attendant must be in the facility at all times when residents are present.

Awake overnight staff for Type B. Because Type B residents may need attention during nighttime sleeping hours, plan for awake overnight coverage in a brain-injury home (confirm the exact staffing subsection for your size/type with HHSC). Texas doesn't set a fixed staff-to-resident ratio — the standard is enough staff to meet residents' needs, 24 hours a day — so document a staffing plan that a surveyor can see meets your residents' acuity.

5.3 Brain-injury competence (build it in)

Texas rules require training on behavior management and fall prevention but do not spell out brain-injury-specific competencies — so make them part of your own training program: cognitive stimulation, personal- and wander-safety, responding to sudden or dysregulated behavior, communication strategies for cognitive/speech impairment, and safe transfers. This is what makes your home genuinely a brain-injury home, and it's what discharge planners and MCOs look for.

5.4 Background checks — the system you must run

Every employee must clear multiple checks before contact with residents, and you re-check some of them every year. Here's the whole system:

- 1. Run a DPS criminal-history check** through the Texas Department of Public Safety on every applicant (statutory basis: Health & Safety Code Chapter 250).
- 2. Add FBI fingerprint checks where required** — fingerprint-based national checks route through DPS/FBI (via IdentoGO). Budget roughly \$38–\$40 per person for the fingerprint components (verify current fee).
- 3. Check the Nurse Aide Registry (NAR) and the Employee Misconduct Registry (EMR)** before hiring AND at least annually — to be sure the person has no finding of abuse, neglect, exploitation, misappropriation, or misconduct. HHSC's online registry search is free.
- 4. Document everything** — keep the dated results in each employee file; a surveyor will ask to see them.

Build the employee file the surveyor will actually open. On a health survey, HHSC pulls staff files to confirm you screened and trained before residents were exposed. For each employee keep, in one folder: the DPS (and FBI, where required) criminal-history result, the dated NAR and EMR search printouts, proof of age/education, the signed orientation record, the on-the-job training log, current first aid/CPR cards, TB screening, and each year's continuing-education certificates. A missing date is treated as a missing check.

5.5 A first-year training calendar (so nothing slips)

The rules attach deadlines to almost every training item; miss one and it becomes a citation. Map them onto a simple calendar the day each person is hired.

- **Before any resident contact:** criminal-history + NAR/EMR clear, and at least 4 hours of orientation.
- **First 16 hours worked:** 16 hours of on-the-job supervision and training for direct-care attendants.
- **Manager, first 3 months:** the 8-hour training on the 26 TAC 553 assisted living standards.
- **Manager, first year:** the HHSC-approved 24-hour management course completed by the first anniversary.
- **Every year, per attendant:** at least 6 documented CE hours including 1 hour fall prevention and 1 hour behavior management, plus a re-run of the NAR/EMR search and current first aid/CPR.
- **Every year, manager:** 12 hours of continuing education.

PART 6

Get Your HHSC Assisted Living License — Step by Step

In short: Apply in the TULIP portal with your manager, policies, and ownership documents; pass the Life Safety Code survey and the health survey; then HHSC issues the license.

6.1 Before you apply

- ☐ Entity formed, EIN, NPI, insurance bound
- ☐ Home secured and brought up to the 26 TAC 553 specs and NFPA 101 Life Safety Code (sprinklers if required)
- ☐ Manager identified and enrolled in the approved 24-hour course
- ☐ Policies & procedures written (your P&P Manual) and forms ready
- ☐ Background-check system in place for staff
- ☐ Completed the required computer-based pre-survey training and saved the certificate

6.2 The application (TULIP)

- 1. Do the pre-survey training first.** HHSC requires computer-based pre-survey/orientation training before you apply; save your completion certificate to submit.
- 2. Apply in TULIP.** The Texas Unified Licensure Information Portal is where you file. The initial application is Form 3720 (initial/relocation/change of ownership).
- 3. Pay the fee.** A three-year license is \$300 base + \$15 per bed, capped at \$2,250. (A two-year option is \$200 + \$10/bed, capped at \$1,500.) If you'll add Alzheimer's certification, that's \$300 more.
- 4. Attach ownership documents** and the disclosures HHSC requires; identify every owner/controlling person.
- 5. Pass the fire-marshall / Life Safety Code inspection.** Demonstrate NFPA 101 compliance for your Type B facility.
- 6. Pass the health (licensure) survey.** This happens with 1–3 residents actually admitted, so HHSC can see care in action.

What HHSC will ask you for:

- Your entity and ownership; the manager's qualifications and course enrollment; your policies & procedures; floor plans and life-safety documentation; proof of the pre-survey training; and your background-check process.

6.3 Timeline

HHSC generally has up to 30 days to review a complete application, you get 30 days to cure any deficiencies, and the license is typically issued within about 45 days of a complete application and passed surveys. An expedited Life Safety Code inspection (about 15 business days) is available, and a 6-month provisional license exists for new construction. Plan for a few months end to end.

Ask before you file. Call HHSC LTCR Policy, Rules & Training at (512) 438-3161 to confirm the current forms, the pre-survey training link, and exactly which attachments your application needs — requirements change, and a complete first submission saves weeks.

PART 7

The Resident's Journey — Referral to Discharge

In short: A STAR+PLUS member is referred by their MCO service coordinator or a hospital discharge planner; you confirm the fit, admit under a residency agreement, deliver and document the service plan, and plan for the least-restrictive setting. Here is the whole journey, stage by stage.

Stage 1 — Referral comes in

A referral comes from a STAR+PLUS MCO service coordinator, or a hospital or rehab (e.g., TIRR) discharge planner (Part 10). Someone calls or emails asking whether you have a bed for a specific person.

What you do:

- Take the basics (age, diagnosis, level of care, funding source, target move date). Ask for the person's current assessment and service plan. Tell them your license type (Type B) and current openings.

Stage 2 — Pre-admission screening (can you safely serve this person?)

Before you say yes, confirm the resident fits a Type B ALF, your capacity, and your abilities. Remember the Type B lines: the resident may need help to evacuate and overnight attention, but must not be permanently bedfast, and their needs can't exceed what an ALF can provide.

What to review:

- Diagnoses and behaviors (aggression, elopement/wandering, swallowing/choking risk); mobility and transfer needs; medications and any special monitoring; and whether your trained staffing can meet all of it safely. When in doubt, ask the MCO service coordinator and the resident's physician.

Stage 3 — The MCO service coordinator sets the plan

For STAR+PLUS members, the MCO's service coordinator — not you — assesses the member, determines they meet the nursing-facility level of care for the HCBS assisted living benefit, and builds the service plan (documented on Form H1700-1). The MCO authorizes the assisted living service and payment on Form H2065-D, and authorizes any add-ons (therapies, adaptive aids) separately.

Stage 4 — Admission & the residency agreement

On move-in, the resident (or their representative) signs a residency agreement covering services, resident rights, fees, who pays room and board, and how residency can end. Give a rights orientation and open a resident record.

The resident record must include:

- Health information and diagnoses; a current medication list with allergies; advance directives / medical power of attorney; physician and MCO service-coordinator contacts; emergency contacts and the representative; and the signed agreement and consents.

Stage 5 — Build the service plan (the care roadmap)

Within your policy timeframe, develop and follow the resident's service plan — the day-to-day plan for THIS person. For a brain-injury resident it typically covers cognitive-stimulation activities, personal- and wander-safety measures, help with the activities of daily living (bathing, dressing, toileting, transfers, eating), medication assistance or administration by the appropriate staff, nutrition and any special diet, and behavior supports. Coordinate it with the physician and the MCO, and have the resident/representative sign off.

Stage 6 — Daily life & care

Deliver the plan: structured daily routines, ADL support, cognitive and social activities (STAR+PLUS AL expects a minimum number of activities per week), community outings as appropriate, three meals a day, and safe medication practices — all mapped to the resident's goals. Maintain your required coverage, including awake overnight staff, and document checks and progress notes so you can show the plan is being followed.

Stage 7 — Reviews, incidents & rights

Review and update the service plan as needs change and whenever the MCO reassesses. Run your quality program with incident reports; protect resident rights (dignity, privacy, the right to complain, health-care directives); and report any suspected abuse, neglect, or exploitation to Adult Protective Services immediately. Everything is documented and followed up.

Stage 8 — Discharge & transition

When a resident's needs change or goals are met, coordinate a safe transition with the MCO and family — to a less-restrictive setting, a higher level of care (if they become permanently bedfast, a nursing facility), or the community — following your termination-of-residency policy and giving proper written notice. Send the receiving provider the records they need, and document the whole transition.

The two forms that anchor a STAR+PLUS admission. Keep the member's Form H1700-1 (the individual service plan the MCO service coordinator builds) and Form H2065-D (the MCO's authorization of the assisted living service and the payment amounts) in the resident record. You cannot bill without a current H2065-D, and a surveyor will expect your own service plan and daily notes to line up with the H1700-1.

7.9 The resident-record checklist (what a surveyor pulls)

On a health survey HHSC opens resident records to confirm the care you're paid for is planned, delivered, and documented. Keep each record complete and current:

For every resident:

- ☐ Signed residency agreement and the resident-rights orientation acknowledgment
- ☐ Diagnoses and health history; the MCO service plan (H1700-1) and authorization (H2065-D)
- ☐ Your own individualized service plan, signed by the resident/representative and reviewed on schedule
- ☐ Current medication list with allergies, and medication assistance/administration records
- ☐ Advance directives / medical power of attorney; physician and MCO service-coordinator contacts
- ☐ Emergency contacts and the legal representative; consents on file
- ☐ Daily/shift documentation, progress notes, and any incident reports with follow-up
- ☐ Weekly activity log showing the STAR+PLUS minimum activities were offered

PART 8

How You Get Paid — STAR+PLUS, Step by Step

In short: Enroll as a Texas Medicaid provider through TMHP/PEMS, contract with the STAR+PLUS health plans, and understand the per-diem. STAR+PLUS pays for the care — never room & board. Verify each resident's coverage before admission.

Summary of the sources:

Source	What it is	Key fact
STAR+PLUS HCBS	Texas Medicaid managed long-term care	Pays the assisted living service (a per-diem); you bill the member's MCO under contract
Private pay	Family/self-pay	Always covers room & board; may cover all care for non-Medicaid residents
CRS (state TBI program)	HHSC Comprehensive Rehabilitation Services	Post-acute TBI rehab — time-limited, sometimes residential; a referral source, not ongoing AL funding
Veterans (VA)	Aid & Attendance, VA residential care	May help fund a veteran's care — verify eligibility

8.1 Enroll as a Texas Medicaid provider (TMHP / PEMS)

1. **Get your NPI first** (Type 2 — Part 3.7).
2. **Enroll in PEMS** — the Provider Enrollment and Management System at tmhp.com. Provide your NPI, your IRS information (EIN/W-9), your HHSC license, and ownership/controlling-interest disclosures.
3. **Complete screening.** Texas Medicaid screens enrollments; facility types may get a site visit. Enrollment/claims help: TMHP 1-800-925-9126.
4. **Revalidate periodically** — you can submit a revalidation up to 180 days before your period ends; keep the enrollment current.

What they'll ask for:

- Your HHSC license type & bed capacity; NPI + tax information; ownership disclosures; and your facility details. A draft PEMS request expires after 180 days, so finish it.

8.2 Contract with the STAR+PLUS health plans (MCOs)

Medicaid enrollment lets you exist in the system; the MCO CONTRACT is what pays you. STAR+PLUS was re-procured effective September 1, 2024, and the current health plans (which vary by service area) are:

- **Statewide/multi-area:** UnitedHealthcare Community Plan of Texas, Molina Healthcare of Texas, Superior HealthPlan (Centene).
 - **Regional:** Wellpoint (formerly Amerigroup) — Lubbock, Nueces, Jefferson, West rural; Community First Health Plans — Bexar (San Antonio); Community Health Choice — Harris (Houston); El Paso Health — El Paso.
 - **Note:** Cigna-HealthSpring is no longer a STAR+PLUS plan. Confirm the current plans for your county on HHSC's "Health Plans by Service Areas" list before you rely on it.
1. Finish Medicaid enrollment first (get your Texas Medicaid provider ID).
 2. Contact each MCO's provider-network / contracting department for the plans in your service area.
 3. Submit their onboarding packet with your W-9, NPI, Medicaid ID, service area, services, and Type B license.
 4. Complete credentialing, then negotiate and sign the provider agreement and rate; get an effective date before you serve or bill members.

8.3 The rate & what's covered

STAR+PLUS HCBS Assisted Living is paid as a per-diem (a daily rate), with Personal Care levels 1, 2, and 3 (level 3 is the heavy-care tier and carries staffing-ratio expectations). HHSC's Provider Finance Department sets the rate

methodology; under managed care the MCO pays you and may negotiate within that framework. The exact per-diem and service code live in the HHSC Provider Finance STAR+PLUS rate schedules — pull the current figure there; don't rely on a guessed number.

Room & board is NOT covered by STAR+PLUS. Medicaid pays only the care/service component. The resident pays room and board from their own income — set by the SSI federal benefit rate minus an \$85 personal-needs allowance — plus any copayment a Medicaid for the Elderly and People with Disabilities (MEPD) specialist calculates. You collect room & board and copay directly from the resident, by a written monthly due date. Build this into your admission and your residency agreement.

How you actually bill (the claims cycle)

1. **Verify eligibility** before and during the stay — check the member's STAR+PLUS enrollment and plan each month; enrollment can change.
2. **Get the authorization** — Form H2065-D from the MCO authorizes the service and documents the payment amounts; confirm the dates and level.
3. **Submit the claim** — electronically to the member's MCO per its provider manual, using your NPI and the authorized service.
4. **Watch timely filing** — traditional Texas Medicaid uses a 95-day deadline; each MCO sets its own filing window in contract, so confirm and bill promptly.
5. **Collect room & board + copay** from the resident by your written due date, separate from the MCO claim.

Ask each plan:

- The per-diem for Type B assisted living and each Personal Care level; how the service coordinator sets the level and what triggers a rate change; the authorization and claims process and portal; the timely-filing window; how room & board / copay is handled; and any enhanced tier for high-acuity brain-injury residents.

8.4 CRS, private pay & veterans

- **CRS (Comprehensive Rehabilitation Services):** the state's program for traumatic brain injury and spinal-cord injury pays for post-acute rehabilitation — which can be residential but is time-limited and aimed at building independence, NOT ongoing assisted living. Treat CRS as a referral pathway and a partner, not a long-term funding source. Apply via 2-1-1 or the HHS Ombudsman 1-877-787-8999.
- **A note on HCS/DBMD:** Texas's HCS and DBMD waivers serve intellectual/developmental disabilities with onset in the developmental period (generally before age 18, or a related condition before age 22). Adult-onset traumatic brain injury usually does NOT qualify — those clients access long-term residential care through STAR+PLUS, not HCS/DBMD.
- **Private pay:** non-Medicaid residents (and every resident's room & board) pay privately — set a clear rate and a written residency/financial agreement; have counsel review it.
- **Veterans:** VA Aid & Attendance (2025–2026 maximum ~\$2,424/mo for a single veteran, more for married veterans; ~\$1,558/mo for a surviving spouse — verify on VA.gov) and VA community residential care may help fund a veteran's care.

PART 9

What It Costs & How Long It Takes

In short: Budget the home and life-safety upgrades, the manager's course, staff training, background checks, and the HHSC fees — and consider accreditation. Plan a few months through licensing, then contracting.

9.1 Startup line items (typical — verify)

Item	What's in it	Ballpark
The home	Lease deposit + first month (or purchase); accessibility mods; furnishings	Varies by market/size
Life-safety upgrades	Fire sprinklers (required for new small ALF), alarms, exits — the big Type B cost	Can be significant — get a bid early
Manager course	HHSC-approved 24-hr course + 8-hr standards training	Several hundred \$
Staff training	Orientation + on-the-job + first aid/CPR per attendant	Per person
Background checks	DPS/FBI fingerprints (~\$38–40 each); NAR/EMR search (free)	Per person
Insurance	GL + professional + abuse + property + auto (+ optional workers' comp), annual	Get quotes
HHSC license	\$300 base + \$15/bed, 3-yr, capped \$2,250	10-bed ≈ \$450 / 3 yrs
Business setup	SOS Form 205 \$300, EIN (free), NPI (free), bank, legal	~\$300 + legal
Policies	P&P Manual + Forms Packet	One-time
Accreditation (optional but recommended)	CARF or Joint Commission (see 9.2)	Low-to-mid five figures over the cycle

9.2 If you pursue accreditation (recommended for brain injury)

- **CARF:** accredits brain-injury programs under its Medical Rehabilitation manual with a Brain Injury specialty designation (inpatient, outpatient, residential, or home-and-community). The survey fee is based on the number of surveyors and survey days plus an annual fee — request a current quote. (888) 281-6531, carf.org.
- **The Joint Commission:** offers Behavioral Health Care & Human Services and Nursing Care Center accreditation; you pay an annual fee across the 3-year cycle plus a survey-year fee, both priced from your services and average daily census. Request a quote: (630) 792-5000, jointcommission.org.
- **It's staged:** both require you to be operating and serving residents before survey, then an initial on-site survey, then a decision (CARF terms run 3-year, 1-year, or provisional; TJC is a 3-year cycle). Plan roughly 12–18 months. Some post-acute brain-injury (CRS) programs require CARF, so it can also open referral doors.

9.3 Timeline

Early	Manager course & fire marshal	Enroll in the approved 24-hr course; confirm the home can meet NFPA 101 / sprinklers
Weeks	Business & home ready	Entity, insurance, home to 26 TAC 553 specs, zoning confirmed
Weeks	Staff & policies ready	Orientation/OJT done, background checks, P&P adopted, pre-survey training
~Months	TULIP application → surveys → license	Apply → Life Safety Code survey → health survey (1–3 residents) → license
Then	Enroll & contract	TMHP/PEMS + STAR+PLUS MCO contracts
Ongoing	Referrals & full census	MCO service coordinators + discharge planners

PART 10

Getting Your First Residents

In short: STAR+PLUS members reach you through their MCO service coordinators, and through hospital and rehab discharge planners — TIRR and the Level 1 trauma centers are the biggest brain-injury pipeline. Build those relationships before you open.

10.1 The referral sources

- **STAR+PLUS MCO service coordinators:** once contracted, each plan's service coordinators place members who need assisted living at the nursing-facility level of care. Tell each MCO's service-coordination team your openings, level, and brain-injury capability.
- **Hospital & rehab discharge planners:** the biggest pipeline for brain injury — the trauma centers and neuro-rehab hospitals below discharge patients who need a Type B home. Introduce yourself, leave a one-pager, and ask how to get on their placement list.
- **The Texas Brain Injury Alliance & HHSC OABI:** the Texas Brain Injury Alliance (txbia.org) connects survivors and families to services; HHSC's Office of Acquired Brain Injury (OABI, 512-706-7191) is the state's ABI resource hub. The national Brain Injury Association helpline is 1-800-444-6443.
- **CRS & ADRCs / 2-1-1:** the state CRS program (via 2-1-1) and the Aging & Disability Resource Centers (1-855-937-2372) route people needing services — get on their radar.

10.2 Trauma & neuro-rehab centers (verify before relying)

Metro	Program (referral source)	Contact (verify)
Houston	TIRR Memorial Hermann — the flagship TX brain-injury rehab	1-800-447-3422; (713) 797-5942
Houston	Memorial Hermann-TMC (Level 1 trauma); Houston Methodist	MH admissions (713) 704-3249; Methodist (713) 790-3311
Dallas	Baylor Scott & White Institute for Rehabilitation (Dallas)	(214) 820-9300
Fort Worth	BSW Institute for Rehabilitation (Fort Worth); JPS (Level 1)	(817) 433-9600; JPS (817) 702-3431
San Antonio	University Hospital (Level 1) + Reeves Rehab; PAM Warm Springs	(210) 358-4000; Warm Springs — verify
Austin	Dell Seton (Level 1); St. David's Rehabilitation Hospital	Verify both numbers before relying
El Paso	UMC of El Paso — Scherr Legate Level 1 Trauma + Rehab	(915) 544-1200

Ask each referral source:

- How do you decide where to place brain-injury patients who need a Type B home? What do you need from me to be on your placement list (license type, capability, availability)? Who is the discharge planner / case manager I should stay in touch with?

PART 11

Who to Call & Exactly What to Ask — Every Verify, in One Place

In short: Your master contact + verification hub. Everywhere the guide says “verify,” the who and the what-to-ask are collected here.

Licensing, the manager & the building

Who	Contact	What to verify / ask
HHSC LTCR — ALF licensing (Policy, Rules & Training)	(512) 438-3161 · hhs.texas.gov (ALF provider pages)	Current forms & fees; the pre-survey training; the TULIP application & attachments; the surveys
HHSC — TULIP portal	hhs.texas.gov (TULIP)	Filing the application (Form 3720); uploading documents
Local Fire Marshal	your jurisdiction	NFPA 101 Life Safety Code for a Type B ALF; sprinkler requirement; the inspection
HHSC-approved manager course / TALA	tala.org	The approved 24-hr course + 8-hr standards training; the manager's CE
City/County Planning & Zoning	your jurisdiction	Allowed use at your address; fair-housing group-home protection — in writing
DPS / IdentoGO + HHSC registries	dps.texas.gov · hhs.texas.gov (EMR/NAR)	Criminal-history & fingerprint process; the NAR and EMR searches

Business, insurance & billing IDs

Who	Contact	What to verify / ask
Texas Secretary of State	(512) 463-5555 · sos.state.tx.us	Forming your LLC (Form 205, \$300); assumed name (Form 503)
Texas Comptroller	comptroller.texas.gov	Franchise tax & Public Information Report for your LLC
IRS	irs.gov (EIN Assistant)	Get your EIN (free)
NPES / CMS	nppes.cms.hhs.gov	Your Type 2 NPI and the right taxonomy
Insurance agent (Trusted Choice)	trustedchoice.com; carriers in Part 3.4	Coverages & limits; abuse & molestation; whether to subscribe to workers' comp
Business bank	Chase/BofA/Wells; Frost Bank; a local credit union	Account fees, limits, business line of credit

Funding & referrals

Who	Contact	What to verify / ask
TMHP — provider enrollment (PEMS)	1-800-925-9126 · tmhp.com	Enrolling your NPI; screening; revalidation
STAR+PLUS health plans (MCOs)	UnitedHealthcare, Molina, Superior, Wellpoint, Community First, Community Health Choice, El Paso Health	Which serve my county; contracting; the Type B per-diem & Personal Care levels; auth & claims; timely filing; room-and-board handling
HHSC Provider Finance (rates)	pfd.hhs.texas.gov	The current STAR+PLUS assisted living per-diem & service code
Medicaid enrollment broker (Maximus)	1-800-964-2777	How members enroll/choose a plan
Accreditor — CARF / TJC	CARF (888) 281-6531 · TJC (630) 792-5000	Cost quote; the operating-first requirement; Brain Injury specialty

Advocacy, the state TBI program & referral partners

Who	Contact	Role
Texas State Long-Term Care Ombudsman	1-800-252-2412 · ltco.texas.gov	Independent resident-rights advocate — post for residents
Adult Protective Services (DFPS)	1-800-252-5400	Mandatory abuse/neglect/exploitation reporting
HHSC Comprehensive Rehab Services (CRS)	2-1-1 · Ombudsman 1-877-787-8999	The state TBI/SCI post-acute program — a referral pathway
Texas Brain Injury Alliance / HHSC OABI	txbia.org · OABI (512) 706-7191 · national 1-800-444-6443	Survivor/family resources; the state ABI resource hub

Aging & Disability Resource Centers	1-855-937-2372	One-stop LTSS information & referral
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You can do this

Opening a Texas brain-injury assisted living home is a real build — a Type B building to the Life Safety Code, a trained manager, trained staff, and vulnerable residents — but it's a defined path, and the Type B license is made for exactly these residents. Take it one rung at a time: start your manager's course and your fire-marshal conversation, form the business, prepare the home and team, get licensed by HHSC, enroll with Texas Medicaid and contract the STAR+PLUS plans, and earn referrals from the MCOs and the trauma centers. Verify each fee and contact as you go, and lean on the people in Part 11.

Reminder. Prepared for PolicyReady.org and grounded in Texas Health & Safety Code Ch. 247, 26 TAC Chapter 553, and current STAR+PLUS/HHSC information as of 2026. This is general information, not legal, tax, or billing advice. Requirements, fees, rates, and contacts change — verify each with HHSC, TMHP, the STAR+PLUS health plans, and qualified counsel before you rely on it. Confirm your payment rate in your provider contract with each STAR+PLUS MCO and check the current HHSC Provider Finance rate schedule.